

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 2, Open-label Study of Encorafenib + Binimetinib in Patients With BRAFV600-mutant Non-small Cell Lung Cancer
Short Title:	An Open-label Study of Encorafenib + Binimetinib in Patients With BRAFV600-mutant Non-small Cell Lung Cancer
Protocol Number:	99482934891306977
NCT Number:	NCT03915951
Sponsor Name:	Pfizer
Investigational Product:	binimetinib
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Percentage of Participants With Confirmed Objective Response (OR) as Determined by Independent Radiology Review (IRR): Objective Response Rate (ORR) was defined as the percentage of participants who had achieved a confirmed best overall response (Complete Response [CR] or Partial Response [PR]) per Response Evaluation Criteria in Solid Tumors (RECIST) version 1.1. Both CR and PR must be confirmed by repeat assessments performed no less than 4 weeks after the criteria for response were first met. CR was defined by the disappearance of all non-lymph node target lesions (where all target lesions were recorded with a length of 0 mm, and any pathological lymph nodes [recorded as target lesion] must have reduction in short axis to <10 mm) and complete disappearance of all non-target lesions (where all non-target lesions were marked "Absent", and all lymph nodes must be non-pathological in size [<10 mm in short axis]). PR was defined by a 30% or more decrease in sum of diameters (SOD) of target lesions, taking as reference the baseline SOD. ORR was calculated with the exact 2-sided Clopper-Pearson 95% CI.
Secondary Obj.:	1. Percentage of Participants With Confirmed Objective Response (OR) by Investigator Assessment 2. Duration of Response (DoR) by IRR and Investigator Assessments 3. Disease Control Rate (DCR) by IRR and Investigator Assessments

2.2 Background & Rationale

This is an open-label, multicenter, non-randomized, Phase 2 study to determine the safety, tolerability and efficacy of encorafenib given in combination with binimetinib in patients with BRAFV600E-mutant metastatic non-small cell lung cancer (NSCLC). Patients who are either treatment-naïve, OR who have received 1) first-line treatment with standard platinum-based chemotherapy, OR 2) first-line treatment with an anti-programmed cell death protein 1 (PD-1)/programmed cell death protein ligand 1 (PD-L1) inhibitor given alone or in combination with platinum-based

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chemotherapy will be enrolled.

3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE2, Status: COMPLETED

Target N: 98

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Histologically confirmed diagnosis of non-small cell lung cancer (NSCLC) that is currently Stage IV.
- * Presence of a BRAFV600E mutation in lung cancer tissue as determined by a local laboratory assay or the presence of other BRAFV600 mutations other than V600E (i.e. K or D) will be considered
- * Patients who are either treatment-naïve (e.g., no prior systemic therapy for advanced/metastatic disease), OR who have received 1) first-line platinum-based chemotherapy OR 2) first-line treatment with an anti-programmed cell death protein 1 (PD-1)/ programmed cell death protein ligand 1(PD-L1) inhibitor given alone or in combination with platinum-based chemotherapy.
- * Presence of measurable disease based on Response Evaluation Criteria in Solid Tumors version 1.1 (RECIST v1.1).
- * Eastern Cooperation Oncology Group (ECOG) performance status of 0 or 1.
- * Adequate bone marrow function characterized by the following at screening:
 - * absolute neutrophil count (ANC) $\geq 1.5 \times 10^9/L$;
 - * Platelets $\geq 100 \times 10^9/L$;
 - * Hemoglobin ≥ 8.5 g/dL (with or without blood transfusions).
- * Adequate hepatic and renal function characterized by the following at screening:
 - * Total bilirubin $\leq 1.5 \times$ upper limit of normal (ULN)
 - * alanine aminotransferase (ALT) and aspartate aminotransferase (AST) $\leq 2.5 \times$ ULN, or $\leq 5 \times$ ULN in presence of liver metastases; Serum creatinine $\leq 1.5 \times$ ULN; or calculated creatinine clearance ≥ 50 mL/min by Cockcroft-Gault formula; or estimated glomerular filtration rate ≥ 50 mL/min/1.73m².

Key Exclusion Criteria:

- * Patients who have documentation of any of the following:
 - * epidermal growth factor receptor (EGFR) mutation
 - * anaplastic lymphoma kinase (ALK) fusion oncogene or
 - * ROS1 rearrangement
- * Patients who have received more than 1 prior line of systemic therapy in the advanced/metastatic setting.
- * Previous treatment with any BRAF inhibitor (e.g., dabrafenib, vemurafenib, XL281/BMS-908662, etc.), or any mitogen-activated protein kinase (MEK) inhibitor (e.g., trametinib, cobimetinib, selumetinib, RDEA119, etc.) prior to screening and enrollment.

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- * Impaired cardiovascular function or clinically significant cardiovascular diseases
- * History of thromboembolic or cerebrovascular events ? 12 weeks prior to the first dose of study treatment. Examples include transient ischemic attacks, cerebrovascular accidents, hemodynamically significant (i.e. massive or sub-massive) deep vein thrombosis or pulmonary emboli.
- * History or current evidence of retinal vein occlusion (RVO) or current risk factors for RVO (e.g., uncontrolled glaucoma or ocular hypertension, history of hyperviscosity or hypercoagulability syndromes); history of retinal degenerative disease.
- * Concurrent neuromuscular disorder that is associated with the potential of elevated creatine (phospho)kinase (CK) (e.g., inflammatory myopathies, muscular dystrophy, amyotrophic lateral sclerosis, spinal muscular atrophy).
- * Patients with symptomatic brain metastasis, leptomeningeal disease or other active central nervous system (CNS) metastases are not eligible.

4.2 Exclusion Criteria

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11. Study Identifiers & Governance

Primary ID:	99482934891306977
Registry Number:	NCT03915951
Sponsor:	Pfizer
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15. Sign-off & Approval

Principal Investigator: _____ Date: _____
Clinical Operations Lead: _____ Date: _____
Lead Statistician: _____ Date: _____